3.3 Status Asthmaticus*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Cannot finish sentences **Objective** • SpO_2 86% • Barely audible wheeze ("silent chest") • HR 130/min, RR 34/min • $PaCO_2$ rising on ABG	**ACTUAL** Impaired gas exchange related to severe bronchospasm and air trapping as evidenced by hypoxemia, a silent chest and rising $PaCO_2$.	• SpO_2 94% or more and ABG improving within 4 hours.	• Call for urgent medical help. • Give high-flow oxygen and back-to-back nebulized bronchodilators as prescribed. • Give IV corticosteroid and IV magnesium sulfate as ordered. • Keep SpO_2, HR, RR and consciousness under continuous watch. • Have intubation equipment ready.	• Achieved when SpO_2 is on target and he can speak. • Drowsiness or exhaustion: prepare for ICU admission and ventilation.
Subjective • "I am too tired to breathe." **Objective** • Accessory muscle use • Sweating heavily • Exhausted	**ACTUAL** Excessive fatigue burden related to increased work of breathing as evidenced by exhaustion and accessory muscle use.	• Less work of breathing within 4 hours.	• Keep him upright and supported. • Take over all care; cut out every non-essential activity. • Watch for signs that the respiratory muscles are tiring.	• Outcome reached when accessory muscle use falls.
Subjective *No specific complaint* **Objective** • High-dose beta-agonist • Systemic corticosteroids	**RISK** Risk for impaired water-electrolyte balance as evidenced by risk factors: high-dose bronchodilator therapy (hypokalemia) and corticosteroid therapy (hyperglycemia).	• Potassium and glucose stay within target.	• Follow serum potassium and blood glucose. • Watch the ECG for dysrhythmia. • Replace electrolytes as ordered.	• Considered met if values remain in range.

* **References for this care plan:** [25], [35], [37], [106], [128]. Full entries are listed in the References section of the CPMN book.